

GOUTNOR 0.5MG TABLET

Composition:

Each tablet contains Colchicine 0.5mg

Description:

A white to off white round shaped tablet with 'C' engraved on one side.

Pharmacodynamics:

Colchicine acts against the inflammatory response to urate crystals, by possibly inhibiting the migration of granulocytes into the inflamed area. This reduces the release of lactic acid and proinflammatory enzymes that occurs during phagocytosis and breaks the cycle that leads to the inflammatory response.

Pharmacokinetics:

Peak plasma concentrations of colchicines are reached within 2 hours of oral administration. Colchicine is partially deacetylated in the liver and unchanged drug and its metabolites are excreted in the bile and undergo intestinal reabsorption. Colchicine is found in high concentration in leucocytes, kidneys and the liver and spleen. Most of the drug is excreted in the faeces but 10% to 20% is excreted in the urine and this proportion rises in patients with liver disorders.

Indication:

For the treatment of acute gout, and for the prophylaxis of recurrent gout and to prevent acute attacks during initial therapy with allopurinol and uricosuric drugs.

Recommended dose:

For oral administration

For treatment of acute gout:

1mg initially then 0.5mg every 2-3 hours until pain is relieved or diarrhoea or vomiting or until a total dose of 6mg is reached.

Do not repeat the course of treatment within 3 days.

For prophylaxis:

0.5mg two or three times daily.

Route of Administration:

Oral administration

Contraindication:

The use of colchicine is contraindicated in

- pregnancy and lactation
- people who are hypersensitive to colchicines
- children under 2 years of age
- blood dyscrasias
- uric acid kidney stones

Colchicine should not be used in patients undergoing haemodialysis since it cannot be removed by dialysis or exchange transfusion.

Warning and precautions:

Colchicine should be given with care to elderly and debilitated patients who may be particularly susceptible to cumulative toxicity; and to those with cardiac, renal, hepatic or gastrointestinal disease.

Interaction with other medicaments:

Potential risk of severe drug interactions, including death, in certain patients treated with colchicines and concomitant P-glycoprotein or strong CYP3A4 inhibitors such as Clarithromycin,

Cyclosporin, Erythromycin, Calcium Channel Antagonists (e.g. Verapamil and Diltiazem), Telithromycin, Ketoconazole, Itraconazole, HIV protease inhibitors and Nefazodone.

P-glycoprotein or strong CYP3A4 inhibitors are not to be used in patients with renal or hepatic impairment who are taking colchicines

A dose reduction or interruption of colchicine treatment should be considered in patients with normal renal and hepatic function if treatment with a P-glycoprotein or strong CYP3A4 inhibitors is required.

Avoid consuming grapefruit and grapefruit juice while using colchicines.

Thiazide diuretics may cause a rise in serum uric acid and this may interfere with the activity of colchicine. Colchicine may impair the absorption of vitamin B12. Requirement for B12 may be increased.

Concomitant use with tolbutamide may lead to colchicine toxicity.

Pregnancy and lactation:

It is contraindicated during pregnancy and lactation

Side effects:

Common side effects include nausea, vomiting, diarrhea and abdominal pain. Larger doses may cause profuse diarrhea, gastrointestinal haemorrhage, muscle weakness, skin rashes, renal and hepatic damage. Dehydration and hypotension may follow. Alopecia, peripheral neuritis and bone marrow depression with agranulocytosis and aplastic anaemia may occur after prolonged treatment. Rarely, peripheral neuritis, myopathy, rhabdomyolysis may occur.

Overdosage:

Overdosage may cause burning feeling in stomach, throat or skin; vomiting; convulsions; diarrhoea, fever, muscle weakness; pulmonary edema; respiratory depression or renal failure. Colchicine should be withdrawn or the dose reduced if adverse gastro-intestinal effects occur. For treatment in acute poisoning, the stomach should be emptied by aspiration and lavage. A purgative such as sodium sulphate 30 gm in 250 ml of water, should be given. Demulcent drinks may be given freely. Morphine sulphate, 10 mg, intramuscularly may be given to relieve abdominal cramps. Atropine has also been given.

Effects on ability to drive and use machines:

No details are available regarding the influence of colchicine on the ability to drive and use machines. However, the possibility of drowsiness and dizziness should be taken into account.

Presentation:

10 tablets per blister. Box of 60, 100, 500 and 1000 tablets.

Storage condition:

Store below 30°C. Protect from light.

Manufacturer and Product Registration Holder:

Noripharma Sdn. Bhd.
Lot 5030, Jalan Teratai,
5 ½ Mile off Jalan Meru,
41050 Klang, Selangor
Malaysia

Date of revision:

November 2022